



OFFICIAL

Delirium Clinical Management Guideline

1. Purpose

This guideline outlines the clinical practice recommendations for the management of patients with delirium in alignment with the [Delirium Clinical Care Standard](#) (ACSQHC, 2021) and evidence-based practice.

2. Applicability

This guideline relates to the care that adult patients (18 years and older) with suspected delirium and adult patients at risk of developing delirium should receive from presentation to hospital through to transition to primary care.³

The care of children and young people (under the age of 18 years) with suspected delirium, and the care of patients with delirium tremens (alcohol or substance withdrawal delirium) are outside the scope of this document.

3. Context

Delirium is an acute change in mental status that is common among many patients in hospital including older patients.⁵ Often a frightening and isolating experience, it is also associated with poor outcomes for patients.⁷ Compared with patients of the same age without delirium, patients with delirium have increased mortality, increased length of stay, increased risk of [hospital acquired complications](#) including falls and pressure injuries, a greater chance of being discharged to a higher dependency of care and a greater chance of developing dementia.^{5,7-8}

The prevalence of delirium in hospitalised patients is reported to range from 14-50%¹⁶, between 32-87% for patients in the Intensive Care Unit (ICU)¹⁷ and from 13-42% in patients admitted to palliative care inpatient units, which may progress to 88% in the last weeks-hours of life.¹⁸ Evidence suggests that approximately 50% of delirium episodes can be reversed, especially those precipitated by medications, infection, and electrolyte abnormalities.¹⁸

While delirium can occur in patients of any age, older patients with cognitive impairment, dementia, severe medical illness or a hip fracture are considered those at greatest risk during a hospital admission.⁷ Sensory impairment (difficulty in hearing or seeing), infection, the use of certain medicines or multiple medicines, abnormal serum sodium levels, urinary catheterisation and depression also predispose older patients to develop delirium.^{5, 21}

Types of delirium

- **Hypoactive** – most common presentations of delirium, but most often missed: often mistaken for depression, patient becomes withdrawn, drowsy or quiet and inactive or sleepy.²⁰
- **Hyperactive** – most recognised: the patient can be loud, restless, have heightened arousal and can be agitated and aggressive.²⁰

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- **Mixed** (hyperactive/hypoactive) – mixed delirium can be more difficult to recognise⁷ as patients present with both hypoactive and hyperactive symptoms with fluctuating levels of activity and lucidity.²⁰
- **Terminal** – terminal restlessness is an agitated delirium that occurs in some patients during the last few days of life.¹⁹

Causes of delirium

The causes of delirium are often diverse and multifactorial. Delirium can develop through a complex interaction between different risk factors and can deteriorate if left undetected and undiagnosed.

Consider using the 'DELIRIUMS' mnemonic for common delirium precipitants:

Causes of delirium:

D – Drugs (withdrawal, toxicity, anticholinergics) / Dehydration

E – Electrolyte imbalance / Environment (stimulation)

L – Level of pain

I – Infection / Inflammation (post-operative)

R – Respiratory failure (hypoxia/hypercapnia)

I – Impaction of faeces / Immobility (including use of restraints)

U – Urinary retention / Use of indwelling catheters

M – Metabolic disorders / Myocardial infarction / Malnutrition

S – Surgery / Sensory impairment / Sleep deprivation

4. Early Identification of Risk

All patients with one or more risk factors for delirium are to have their cognition screened on presentation and/or admission to the hospital using the 4AT, Abbreviated Mental Test Score (AMTS), and the Single Question in Delirium (SQiD).

4.1 Risk factors for delirium

Patients with one or more of the following risk factors should receive cognitive screening:

- Age >65 years or >45 years for Aboriginal and Torres Strait Islander peoples³
- Known cognitive impairment or diagnosed dementia
- History of Delirium
- Severe medical illness or risk of deterioration
- Current hip fracture
- Family and/or carer concerns regarding mental state
- Cognitive concerns raised by others (including healthcare workers).
- Acute neurological condition

4.2 Single Question in Delirium

The Single Question in Delirium (SQiD) is tool for early recognition of delirium to be asked of patient's friends or relatives on a regular basis.¹²

Ask a family or staff member for all patients on presentation / admission:

"Do you think that [patient's name] has been more confused, sleepy or drowsy lately?"

If yes, the patient is considered 'SQiD positive' and should be further assessed by the treating team.

4.3 4AT

The 4AT is a rapid screening tool designed for the assessment of delirium and cognitive impairment (Appendix 1). The 4AT consists of the following four items:

- Alertness – evaluates the level of alertness.
- AMT4 – abbreviated mental test that assesses cognitive function.
- Attention – tests the patient's ability to focus on specific tasks.
- Acute Change – assess the acute change in cognition or fluctuation.

If the score is 4 or more the nurse will refer immediately to the Medical Officer for further delirium assessment (to be undertaken within 2 hours) and commence a behavioural observation form (BOF).

If the score is 1-3 the nurse will refer to the Occupational Therapist (OT) for further review (to be undertaken within 48hrs).

The following [Take 5 for the 4AT](#) has been developed.

4.4 Abbreviated Mental Test Score

If a patient is 'SQiD positive' or has one or more risk factors for delirium, the 4AT should be completed, also an Abbreviated Mental Test Score (AMTS) can be completed with evaluation for reversible causes of altered mental status in patients deemed to have abnormal cognition.

The AMTS is a 10-point rapid assessment for cognitive impairment. It can be performed by any health care professional experienced in assessing AMTS or under the direct supervision of an experienced clinician. The AMT is available as a sticker (shown below) to be completed and included in the patient's integrated progress notes.

ABBREVIATED MENTAL TEST SCORE			
Each question scores one mark			
Name _____		Date _____	
1. Age	0/1	6. Recognition of two persons	
2. Time (to nearest hour)	0/1	(eg Doctor, Nurse, Magazine photo etc).	0/1
3. Address: For recall at end of test, should be repeated by patient to ensure it has been heard correctly.		7. Date of Birth	0/1
42 West Street	0/1	8. Year of first or second World War	0/1
4. Year	0/1	9. Name of present P.M.	0/1
5. Name of Hospital	0/1	10. Count backwards - twenty to one	0/1
SIGNATURE _____		SCORE _____	

If AMTS is <8 or the patient is SQiD positive, this suggests cognitive impairment:

- Inform the Shift Coordinator and treating team Medical Officer
- Liaise with appropriately trained health professional to perform further cognitive assessment.

If the AMTS is ≥8 continue to monitor the patient and be alert to changes in the patient's thinking or behaviour. If changes are observed, rescreen, inform the Medical Officer, support the patient with person centred approaches and document the changes in the healthcare record.

5. Interventions to prevent delirium

Continue daily observations of the patient's ability to perform their own activities of daily living, mobilise and initiate self-care as appropriate.

Prevention strategies should be commenced to manage delirium risk factors such as addressing and monitoring:

- Pain
- Infection
- Nutrition
- Constipation
- Hydration
- Immobility
- Sleep disturbance
- Multiple medications
- Environment

Refer to [Appendix 4 – Delirium prevention strategies](#).

Remain ALERT to patients at risk of delirium. If changes are observed, rescreen, inform the Shift Coordinator and refer to treating team for further assessment. Complete a risk re-assessment in the Multidisciplinary Comprehensive Care Plan (MDT Comprehensive Care Plan).

6. Patient Centred Information and Support

Where possible, family members and carers of people at risk of delirium should be involved in identifying any changes/fluctuations in behaviour and thinking.

Carer's information is essential to determine if there has been:

- An acute decline (characteristic of delirium) or
- A much longer decline (characteristic of dementia).

In addition to asking the SQiD, the clinician should:

- Acknowledge the family and/or carers expertise and contributions
- Be aware of common statements that someone is at risk and requires further assessment. These can sometimes be said in passing, for example, carer comments may include "Pleasantly confused" / "Not coping" / "Not themselves" / "Different to normal", "Withdrawn" / "This is not my mum".
- Work with the patient, family and/or carer to determine person-centred information and deliver care tailored to the individual's needs
- Offer the family and/or carer the opportunity to speak with the Social Worker or a Dementia Change Champion for ongoing support whilst the patient is in hospital, and for ongoing support in the community
- Provide the family and/or carer with a [Delirium Patient Information Leaflet](#) and the 'Prepare to Care' resource pack from Carers Australia WA.

Posters are also available for the wards to encourage carers and family members to raise their concerns if they observe changes in the patient's level of confusion, thinking or behaviour. These can be found on the [Dementia and Delirium Information Hub](#) page.

7. Assessing and Diagnosing Delirium

If cognitive screen scores suggest cognitive impairment, clinical escalation is required to the Shift Coordinator and Medical Officer. Record 4AT, AMTS, and SQiD results in patients' notes and inter-ward transfer summary within the Patients Care Plan.

Delirium assessment – The Confusion Assessment Method

The [Confusion Assessment Method](#) (CAM) is a validated diagnostic tool that allows trained clinicians to diagnose delirium.

The CAM tool assesses the presence, severity, and fluctuation of nine delirium features: acute onset, inattention, disorganized thinking, altered level of consciousness, disorientation, memory impairment, perceptual disturbances, psychomotor agitation or retardation, and altered sleep–wake cycle.

Whilst trained clinicians are responsible for using the CAM to diagnose delirium, all staff should be aware of its four clinical features:

- A – Acute onset and fluctuating course
- B – Inattention
- C – Altered level of consciousness
- D – Disorganised thought processes

A diagnosis of delirium according to the CAM requires the presence of features A, B, and either C or D – refer to [Appendix 2](#).

The CAM should be used:

- When admission screening identifies probable cognitive impairment
- In response to any changes observed in a patient's thinking, behaviour or physical function
- When a family member, carer or staff member raise concerns e.g. "this is not my mum", "there's something different about my patient".
- In critical care areas or in the recovery room after surgery, the CAM for nonverbal (ventilated) patients (CAM-ICU) should be used – refer to [Appendix 3](#).

Cognitive Assessment

Further formal cognitive assessment should be conducted by appropriately trained Medical Officers, Occupational Therapists, or psychiatric team. Where possible assessment should include use of a validated assessment tool. Commonly used tools include:

- **Mini-mental state exam (MMSE)** – widely used cognitive test for dementia in clinical practice. The MMSE is scored on a 30-point scale, with items that screens orientation (temporal and spatial; 10 points), memory (registration and recall; 6 points), attention/concentration (5 points), language (verbal and written; 8 points), and visuospatial function (1 point)
- **Montreal Cognitive Assessment (MoCA)** – a validated tool scored on a 30-point scale, with items that screens delayed word recall (5 points), visuospatial/executive function (7 points; includes clock-drawing), language (6 points), attention/concentration (6 points), and orientation (6 points). It may help predict dementia in people with Mild Cognitive Impairment (MCI), and because it tests for executive function, it is more sensitive in this regard than the MMSE.

- **Rowland Universal Dementia Scale (RUDAS)** – a brief cognitive assessment tool that was created for culturally and linguistically diverse populations. It is used to both rule in (positive likelihood ratio 5.5) and rule out (negative likelihood ratio 0.27) dementia. It has a particularly high specificity across cultures in immigrant populations (91.4%). The RUDAS performed similarly to the MMSE, and results may be less affected by education and language.
- For Aboriginal and Torres Strait Islander patients a more culturally sensitive screen is available called the **Kimberley Indigenous Cognitive Assessment (KICA)**. The KICA Carer Tool is to be used when the KICA Cog is unable to be used for a patient. These tools can be downloaded from www.wacha.org.au/kica.html

8. Identifying and treating underlying causes: A comprehensive care approach

Once cognitive impairment is identified and / or delirium is confirmed by the medical officer, manage medical issues and symptoms, including pain. In collaboration with the multidisciplinary team and in partnership with the patient, family and carer:

- Develop and implement an individualised and integrated prevention and management plan to reduce risks and optimise recovery and wellbeing
- Include Goals of Patient Care
- If unable to obtain any supportive information from the patient directly, liaise with their relative/carer/significant other, to ensure person centred strategies are established
- Document these in the integrated notes / TOP 5 form
- Refer on to appropriate inpatient specialist services as required e.g. Gerontology, Psychiatry, Aboriginal Health Liaison Officer, Dietetics and Nutrition etc.
- Liaise with ward Dementia Change Champions and AKG Dementia Lead

8.1 Person-centred care

- Determine personal information that may help calm and orient patient e.g. the names of family and friends, hobbies, significant events
- As appropriate, encourage family/carers to visit and bring in patient's personal and familiar objects (photographs, clothes, radio or CD/tape player with favourite music etc.)
- Utilise the TOP5 tool to document and share these strategies with the wider team.

8.2 Mobility and Occupation

- Encourage mobility and independence with all activities of daily living
- Engage with MDT: including Physiotherapist and Occupational Therapist and update the Patient Care Plan/FRAMP as necessary
- Ensure patient wears appropriate footwear, does not walk in socks/anti-embolic stockings
- Consider using resources from the activity box and referral to the "Forget Me Not" Volunteers (as available) on the ward.

8.3 Communication

- Provide communication aides as appropriate e.g. pen and paper for hearing impaired with speech impairment/tracheostomy – liaise with Speech Pathology as necessary

- Refer to the Dementia Australia resource '[Let's Talk](#)' for communication tips for talking with people with dementia.
- Provide access to a radio as appropriate for visually impaired patients.

8.4 Environment

- Minimise room/ward changes
- Optimise orientation: increase visibility of clocks and orientation boards.
- Reduce environmental clutter (noise/light/furniture) to minimize stimulation, hazards/falls
NB: Mobility aids are not considered clutter once issued to a patient
- If patient is allocated a ensure clinical handover is provided. Refer to [Communicating for Safety Procedure](#).
- Respond to changes in behaviour using non-pharmacological strategies
- Offer the patient emotional support and other non-drug strategies, involving carers where possible to support the patient and de-escalate any distress
- Avoid using restraints or bed rails for patients with delirium as they can increase agitation, prolong delirium and increase the risk of injury – refer to [Safe Use of Bedrails Guideline](#), [Restraint](#) and [Seclusion](#) Policies
- Protect invasive lines/feeding tubes and remove at the earliest opportunity in consultation with the treating team Medical Officer. Consider the use of busy blankets, available from the Occupational Therapists.
- Liaise with area Occupational Therapist and Physiotherapists if indicated for individualised environmental assessment

'Forget Me Not' - Where you see the 'Forget Me Not' symbol, you will find a person or resource to support you in delivering a better way to care for patients with delirium and/or dementia across AKG.

8.5 Falls

Refer to the Falls Prevention and Management Policy for further details.

- If the patient assessed as a falls risk, keep bed at lowest level and against the wall (where possible) when staff not in attendance.
- Ensure Falls Alarm matt in place (where available and in working order).

9. Preventing Complications of Care

Patients with delirium have a higher rate of hospital acquired complications compared with patients of the same age without delirium. It is therefore important to complete a comprehensive risk screening and assessment for the following:

- Carry out a comprehensive assessment of the patient, in consultation with the patient and their carer, to identify possible causes for delirium; including a medication review
- Assess for falls risk – postural hypotension is a common side effect of medications used to manage delirium, increasing the risk of falls. Refer to the [Fall and Fall Injury Prevention and Management Policy](#)
- Assess for pressure injuries in accordance with the [Pressure Injury Prevention and Management Policy](#) – hypoactive delirium and poor mobility increases the risk for pressure injury

- Assessment and correction of dehydration, malnutrition and constipation – refer to the [Adult Malnutrition Diagnosis and Documentation Procedure](#) and [Nutrition and Hydration Policy](#)
- Pain assessment and management using the appropriate validated tools – refer to the [Pain Management Procedure](#)
- Ensure bladder is not full. Correct urinary incontinence (seen in over 50% of delirious patients)²² and screen for urinary tract infection – refer to the [Bladder Management: Continence Procedure](#).

10. Clinical communication

Information exchange during any clinical handover process must adhere to the [Communicating for Safety Procedure](#) requirements using the ISoBAR framework.

Clinical handover

Critical information or risks about a patient are communicated in a timely manner to clinicians who can make decisions about the care. The critical information pertinent to this policy includes the following:

- Handover processes should identify patients who have been identified as having cognitive impairment and include communication of information relevant to their management. This information must be documented, and the person receiving the handover needs to review this documentation and ensure that it is understood
- The most recent cognitive screening and assessments scores should be reported at every clinical handover including any implemented and pending strategies to mitigate the impact of the delirium, including TOP5.
 - Use objective language e.g. “Most recent AMT score 8/10 but carer concerns and SQiD positive”. Avoid phrases like “pleasantly confused” or “poor historian”. Example of objective language: “Not at usual cognitive baseline”.

11. Avoiding Use of antipsychotic medicines

- Minimise use of antipsychotics and benzodiazepines as much as possible.
- Treatment with antipsychotics can be considered if a patient with delirium is distressed and the cause of their distress cannot be addressed with the above non-drug strategies.
- Also refer to ETG for guiding principles: [Topic | Therapeutic Guidelines \(health.wa.gov.au\)](#)
- Refer to: [Management of Agitation in the Older Patients](#)
- Refer to: [Antipsychotic Polypharmacy and High Dose Antipsychotic Prescribing Procedure](#)
- Discuss with the patient and carer the choice of antipsychotic medicine, the risks and benefits, dosage, and duration.

12. Transition from hospital care

Before a patient with current or resolved delirium leaves hospital, the patient and their carer are involved in the development of an individualised care plan and are provided with information about delirium.

Effective communication between hospital clinicians and ongoing clinical providers is essential for the ongoing care and recovery of patients with delirium, many of whom may have unresolved symptoms at the time of discharge.

The following should be included in the discharge summary for patients being discharged with current or resolved delirium:

- The most recent cognitive scores and episodes of delirium
- Strategies for managing persistent delirium and for preventing delirium recurrence
- Describe all ongoing treatments and any follow up needed for any comorbidities
- List all medicines that the patient needs to take, specifying for each one:
 - generic drug name
 - dose
 - reason for use, and
 - duration.
- Explain why any medicines have been stopped or changed.
- If an antipsychotic medicine is prescribed, include a plan for ongoing review and withdrawal.
- Include contact details of ongoing support services available in the community, as appropriate.
- Referral to memory clinic confirmed (if required).
- Provide the discharge summary to the patient and their carer before they leave hospital and to their general practitioner and other ongoing clinical providers within 48 hours of the patient leaving hospital.

Occupational therapy to have reviewed patient prior to discharging home to assess if a home environment review is indicated. If a patient is discharging to residential aged care, consider the referral to CO-HIVE.

13. Workforce training and support resources

Clinicians working in relevant clinical areas are to be orientated to delirium resources by their line manager.

Resources available to the workforce include:

- [Delirium and Dementia Information](#) page
- [Take 5 Cognitive Impairment, Delirium and Dementia Presentations](#)
 - Detecting Delirium
 - Agitation in the Older Person
 - The CAM – Assessment Tool for Delirium
 - De-escalation in Cognitive Impairment
 - Less Drugs, More Hugs
- [Delirium eLearning package](#) (WA Centre for Health Aging)
- Understanding Delirium and Dementia Study Day – refer to the [Education and Professional Development Information Hub page](#)
- Knowledge of location and contents of site-specific delirium activity resources e.g. activity box/busy blankets

- Dementia Change Champions and Dementia Lead
- [Management of Agitation in the Older Patients](#)

14. Compliance monitoring and evaluation

Compliance monitoring and evaluation of the requirements of this policy will be monitored by the Cognitive Impairment Sub-Committee under the oversight from the Comprehensive Care Steering Committee. This will include a combination of CoBRA rolling audit results (indicator 1b and 4a), annual retrospective documentation audits (5a, 5b, 6a, 7, 8a) and database analysis (6b & 8b). Results will be provided to clinicians via respective committees and to the hospital board via the Comprehensive Care Committee.

Indicator 1b: Proportion of admitted patients aged 65 years or older or 45 years or older for Aboriginal and Torres Strait Islander people who were screened for cognitive impairment using a validated tool within 24 hours of presentation to hospital.

• **Indicator 4a:** Proportion of admitted patients who screened positive for cognitive impairment on presentation to hospital who were then assessed for delirium using a validated tool.

• **Indicator 5a:** Proportion of patients with delirium who had a comprehensive assessment that includes relevant multidisciplinary consultation to investigate the cause(s) of delirium.

Indicator 5b: Proportion of patients with delirium who received multicomponent interventions to treat delirium.

• **Indicator 6a:** Proportion of patients with delirium who were assessed for risk of functional decline, dehydration, malnutrition, falls and pressure injuries.

• **Indicator 6b:** Proportion of patients with delirium who experienced dehydration, malnutrition, a fall resulting in fracture or other intracranial injury or a pressure injury during their hospital stay.

• **Indicator 7:** Proportion of patients with delirium who were prescribed antipsychotic medicines in hospital.

• **Indicator 8a:** Proportion of patients with current or resolved delirium who had an individualised comprehensive care plan on discharge.

• **Indicator 8b:** Proportion of patients aged 65 years or older or 45 years or older for Aboriginal and Torres Strait Islander people who experienced delirium in hospital and were readmitted for delirium within 10 days.

- Monitoring of hospital acquired complication trended data to identify rates of delirium among acute admitted patients and those readmitted within 28 days with delirium.

15. Legislative context

The following documents are required to give effect to this policy:

- HDWA
 - [Recognising and Responding to Acute Deterioration Policy MP 171/22](#)
 - [Consent to Treatment Policy MP 0175/22](#)
 - [Medication Review Policy MP0104/19](#)
- AKG
 - [Management of Agitation in the Older Patients](#)
 - [Antipsychotic Polypharmacy and High Dose Antipsychotic Prescribing Procedure](#)

16. Supporting information

The following documents support the implementation of this policy:

- HDWA
 - [Recognising and Responding to Acute Deterioration Policy – Supporting information](#)
- AKG
 - [Fall and Fall Injury Prevention and Management Policy](#)
 - [Dietetics Malnutrition Diagnosis and Documentation](#)
 - [Communicating for Safety Procedure.](#)
 - [Mental Health Restraint Policy](#)
 - [Mental Health Seclusion Policy](#)
 - [Pain Management Procedure](#)
 - [Safe Use of Bedrails Guideline](#)
 - [Nutrition and Hydration Policy](#)
 - [Bladder Management – Continence Procedure](#)
- RPBG
 - [Take 5 – Cognitive Impairment](#)

17. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (2nd edition):

- Standard 2 – Action 2.6: The health service has processes for clinicians to partner with patients and/or their substitute decision-maker to plan, communicate, set goals and make decisions about their current and future care.
- Standard 5 – Action 5.10: Clinicians use relevant screening processes.
- Standard 5 – Action 5.14: The workforce, patients, carers and families work in partnership.
- Standard 5 – Action 5.29: The health service organisation providing services to patients who have cognitive impairment or are at risk of developing delirium has a system for caring for patients with cognitive impairment.
- Standard 6 – Action 6.9: Clinicians and multidisciplinary teams use clinical communication processes to effectively communicate critical information, alerts and risks, in a timely way, when they emerge or change.
- Standard 8 – Action 8.5: The health service organisation has processes for clinicians to recognise acute deterioration in mental state.

[Delirium Clinical Care Standard](#)

18. References

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19. Glossary

The following definitions are relevant to this policy.

Term	Definition
Carers	People who provide care and support to family members and friends who have a disease, disability, mental illness, chronic condition, terminal illness or general frailty.
Cognition	The mental activities associated with thinking, learning and memory.
Cognitive impairment	Difficulty with memory, thinking, concentration and ability to read and write.
Delirium	A disturbance of consciousness, attention, cognition and perception that develops over a short period of time (usually hours or days) and tends to fluctuate during the day.

20. Document owner

Director Allied Health & Ambulatory Care

Enquiries relating to this guideline may be directed to:

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21. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	04 12 2025	04 12 2028	Full Review, significant changes throughout document.

22. Authorisation

This guideline has been authorised and issued by:

Authorised by	Danielle Kilmurray - Director Allied Health & Ambulatory Care
Authorisation date	03 12 2025
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23. Appendices

23.1 Appendix 1- 4AT



Assessment test for delirium & cognitive impairment

Patient name: (label)

Date of birth:

Patient number:

Date:

Time:

Tester:

CIRCLE

[1] ALERTNESS

This includes patients who may be markedly drowsy (eg. difficult to rouse and/or obviously sleepy during assessment) or agitated/hyperactive. Observe the patient. If asleep, attempt to wake with speech or gentle touch on shoulder. Ask the patient to state their name and address to assist rating.

Normal (fully alert, but not agitated, throughout assessment)	0
Mild sleepiness for <10 seconds after waking, then normal	0
Clearly abnormal	4

[2] AMT4

Age, date of birth, place (name of the hospital or building), current year.

No mistakes	0
1 mistake	1
2 or more mistakes/untestable	2

[3] ATTENTION

*Ask the patient: "Please tell me the months of the year in backwards order, starting at December."
To assist initial understanding one prompt of "what is the month before December?" is permitted.*

Months of the year backwards	Achieves 7 months or more correctly	0
	Starts but scores <7 months / refuses to start	1
	Untestable (cannot start because unwell, drowsy, inattentive)	2

[4] ACUTE CHANGE OR FLUCTUATING COURSE

Evidence of significant change or fluctuation in: alertness, cognition, other mental function (eg. paranoia, hallucinations) arising over the last 2 weeks and still evident in last 24hrs

No	0
Yes	4

4 or above: possible delirium +/- cognitive impairment
1-3: possible cognitive impairment
0: delirium or severe cognitive impairment unlikely (but delirium still possible if [4] information incomplete)

4AT SCORE

GUIDANCE NOTES

Version 1.2. Information and download: www.the4AT.com

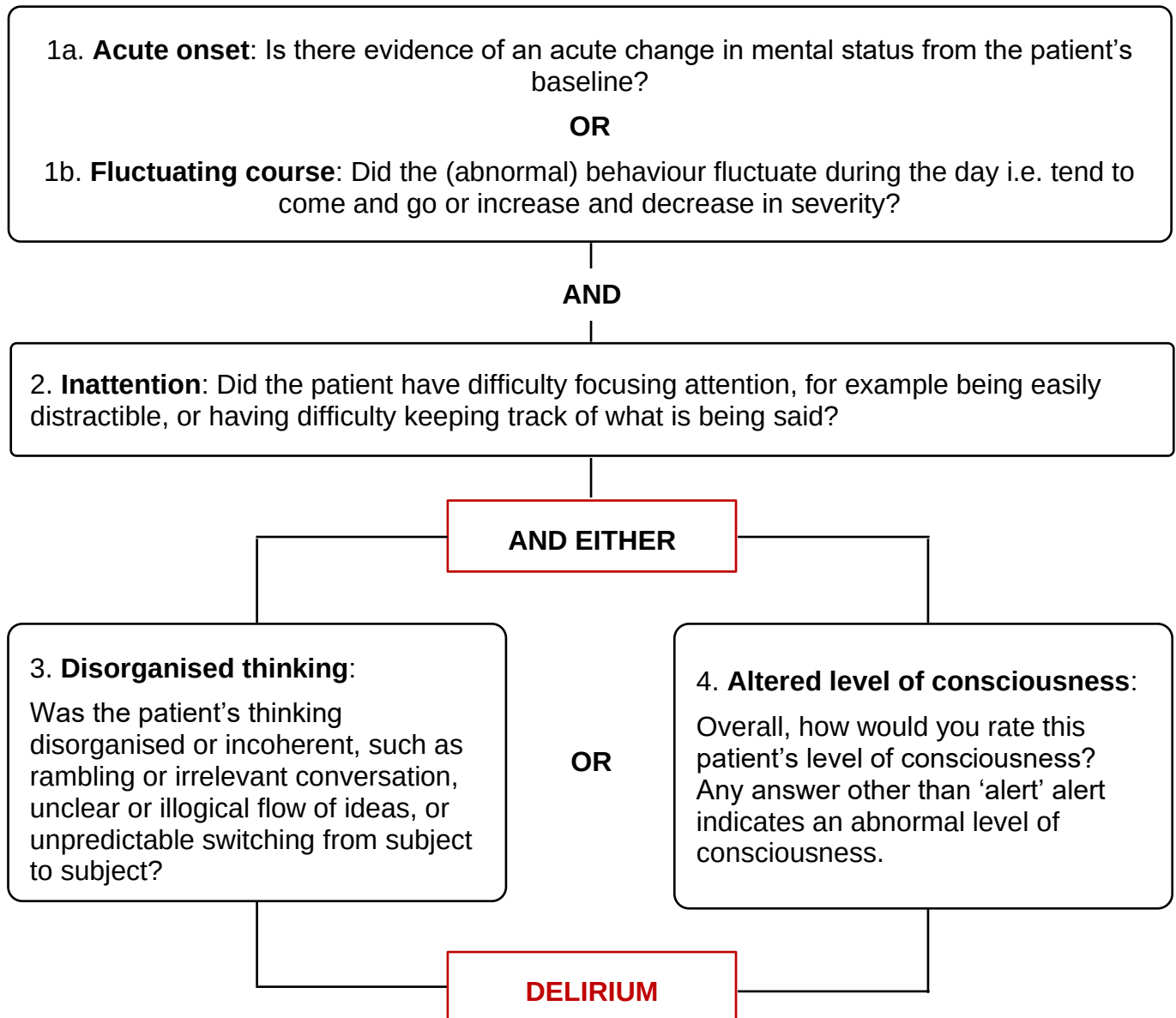
The 4AT is a screening instrument designed for rapid initial assessment of delirium and cognitive impairment. A score of 4 or more suggests delirium but is not diagnostic: more detailed assessment of mental status may be required to reach a diagnosis. A score of 1-3 suggests cognitive impairment and more detailed cognitive testing and informant history-taking are required. A score of 0 does not definitively exclude delirium or cognitive impairment: more detailed testing may be required depending on the clinical context. Items 1-3 are rated solely on observation of the patient at the time of assessment. Item 4 requires information from one or more source(s), eg. your own knowledge of the patient, other staff who know the patient (eg. ward nurses), GP letter, case notes, carers. The tester should take account of communication difficulties (hearing impairment, dysphasia, lack of common language) when carrying out the test and interpreting the score.

Alertness: Altered level of alertness is very likely to be delirium in general hospital settings. If the patient shows significant altered alertness during the bedside assessment, score 4 for this item. **AMT4 (Abbreviated Mental Test - 4):** This score can be extracted from items in the AMT10 if the latter is done immediately before. **Acute Change or Fluctuating Course:** Fluctuation can occur without delirium in some cases of dementia, but marked fluctuation usually indicates delirium. To help elicit any hallucinations and/or paranoid thoughts ask the patient questions such as, "Are you concerned about anything going on here?"; "Do you feel frightened by anything or anyone?"; "Have you been seeing or hearing anything unusual?"

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23.2 Appendix 2 – Confusion Assessment Method

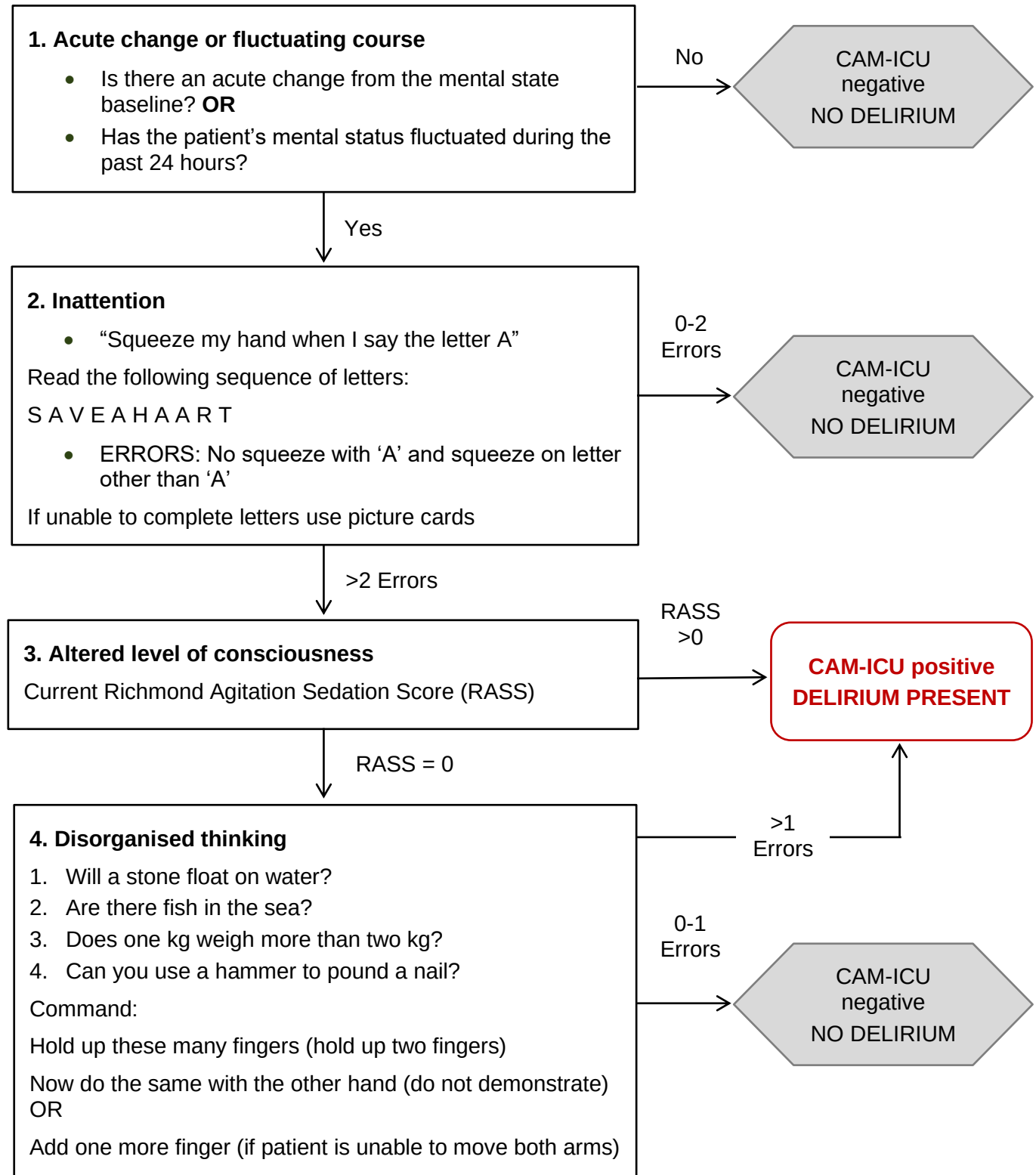
Confusion Assessment Method (CAM) criteria: at least one criterion on each of the three rows must be met for a positive result for the presence of delirium.



Adapted for use at AKG from: Confusion Assessment Method. Copyright 2003, Hospital Elder Life Program, LLC.
Available at URL: www.hospitalelderlifeprogram.org

23.3 Appendix 3 – Confusion Assessment Method for the ICU

Where the patient is non-verbal the Confusion Assessment Method for the Intensive Care Unit (CAM-ICU) should be used as per the following algorithm:



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23.4 Appendix 4 – Delirium prevention strategies

The following strategies should be considered in the development of an individualised and integrated prevention and management plan to reduce risks and optimise recovery and wellbeing for patients at risk of delirium:

Risk Factor	Prevention strategy
Pain	- Assess for pain - Look for non-verbal signs of pain, particularly in those with communication difficulties (for example, people with learning difficulties or dementia, or people on a ventilator or who have a tracheostomy).
Infection	- Look for and treat infection (sepsis pathway, urinalysis etc.) - Avoid unnecessary catheterisation.
Nutrition	- Assist and support the person to eat well - Find out about individual food preferences and routines - If patient has dentures, ensure they fit properly - Allow adequate time for completion of meals.
Constipation	- Assess and manage bowel function - Encourage fibre and fluids - Allow privacy and time to use the toilet or commode.
Hydration	- Ensure adequate fluid intake to prevent dehydration by encouraging the person to drink – consider offering subcutaneous or intravenous fluids if necessary - Seek advice if necessary when managing fluid balance in people with comorbidities (e.g. heart failure or chronic kidney disease).
Sensory impairment	- Resolve any reversible cause of the impairment e.g. impacted ear wax - Ensure hearing and visual aids are available to and used by people who need them, and that they are in good working order.
Sleep hygiene	- Avoid nursing or medical procedures during sleeping hours, if possible - Schedule medication rounds to avoid disturbing sleep - Reduce noise to a minimum during sleep periods.
Medication	- The risk of delirium increases with specific medications such as opioids, benzodiazepines, and possibly with antihistamines - Consult with Pharmacist for medication review.
Mobility	- Encourage people to: - mobilise soon after surgery - walk (provide appropriate walking aids if needed – these should always be accessible) - Encourage all people, including those unable to walk, to carry out active range-of-motion exercises.
Environment	Provide appropriate lighting and clear signage; a clock (consider providing a 24-hour clock in critical care) and a calendar should also be easily visible to the person at risk

Risk Factor	Prevention strategy
	• Talk to the person to reorientate them by explaining where they are, who they are, and what your role is • Introducing cognitively stimulating activities • Facilitate regular visits from family and friends.